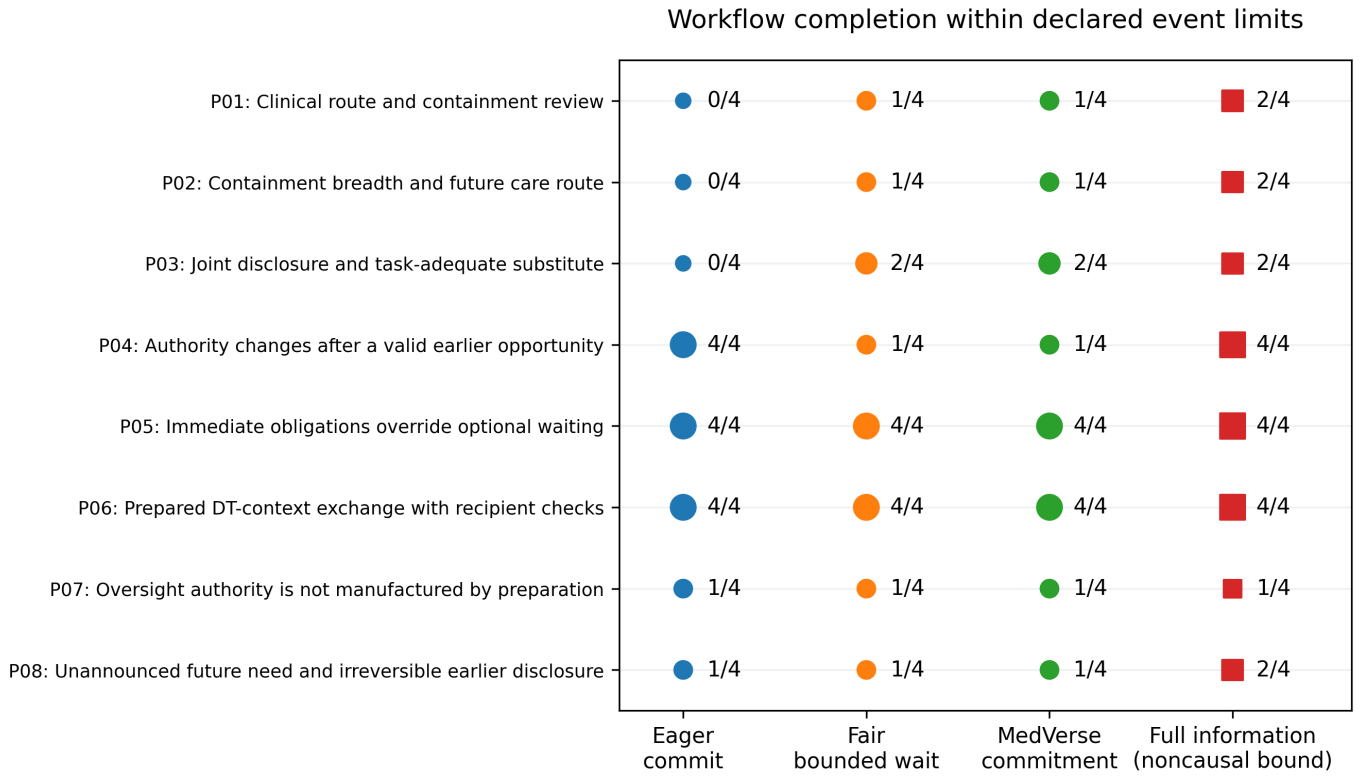
# Team MedVerse | Gate 32

## Bounded commitment: measured benefits and retained costs

A causal controller decides COMMIT, WAIT or authorized PREPARE from released information. The original selector and authorization validator remain unchanged. Eight modeled prefix families supply 32 correlated continuation branches; 21 admit a complete solution with full hindsight.

| Controller | Complete (all branches) | Complete (feasible subset) | Clinical on time* | Mean wait steps** | Invalid commits |
|---|---|---|---|---|---|
| Eager | 14/32 | 14/21 | 28/30 | 0.00 | 0 |
| Fair bounded wait | 15/32 | 15/21 | 29/30 | 1.19 | 0 |
| MedVerse commitment | 15/32 | 15/21 | 29/30 | 1.19 | 0 |

* Clinical denominator: 30 branches where the reference can meet the modeled obligation. ** Proposal-event waiting, not clinical waiting time. Full-information feasibility is a noncausal diagnostic bound, never an online input.




Eight internally authored prefix families; four correlated continuations per family.
The full-information column is an evaluation-only upper bound, not a deployable baseline.

**Interpretation.** Four continuations improve versus eager commitment; three lose an earlier valid opportunity before revocation. Fair bounded wait ties MedVerse on all 32 continuations. No global waiting policy or unique algorithmic superiority is established.

**Integrity.** 52/52 development checks; 177/177 same-history comparisons; 72/72 nonvacuous change-before-commit probes; 432/432 prior event outcomes preserved. C01/C02 remain unchanged because their original inputs provide no visible review window. Clinical/legal review remains pending.

Source: 20261104T013623_796085Z | Deterministic repeat and isolated-runtime checks agree.